

INVESTOR AUDIENCE

Acuity Engine

A healthcare operations software platform that helps hospitals build safer, fairer, and more defensible assignments while creating a repeatable subscription and expansion revenue model.

Clinical Ops

Balances patient assignments using staffing, acuity, discharge risk, continuity, report-source burden, and room spread.

Business Model

Implementation fee plus recurring subscription revenue, with optional enterprise rollout across a regional hospital footprint.

WHY THIS IS COMMERCIALLY ATTRACTIVE**The pain is frequent, costly, and still under-digitized**

- Assignments are still often built on paper or in Excel while charge staff are being interrupted constantly.
- That process increases the risk of hidden workload concentration, excess handoffs, and inconsistent shift-to-shift assignment quality.
- The financial exposure is not only operational delay. It extends to turnover, burnout pressure, and avoidable hospital-acquired events.

\$61,110

Average bedside RN replacement cost used in current workforce reporting.

\$289K

Estimated annual value of a 1% RN turnover improvement for the average hospital.

WHAT CUSTOMERS BUY

Implementation revenue plus recurring workflow revenue

Implementation package

- Workflow discovery and configuration
- Staffing and patient-detail setup
- Role-based rollout and training
- Oncoming/live assignment launch support

Recurring platform subscription

- Assignment engine and rebalance logic
- Shared live/oncoming boards
- Reporting, handoff, and metrics views
- Continuous product improvement and support

Pricing below is a proposed commercial model inferred from workflow value, implementation burden, and hospital savings potential. It is not presented as a historical contract dataset.

PRICING STRATEGY

Illustrative commercial packaging

Package	Target	Implementation fee	Monthly recurring	Annual recurring
Pilot	1 hospital	\$40,000	\$8,000	\$96,000
Regional	3 hospitals	\$100,000	\$20,000	\$240,000
System	5 hospitals	\$150,000	\$32,000	\$384,000
Area Network	10 hospitals	\$250,000	\$60,000	\$720,000

DEPLOYMENT STORY**Fast enough to support enterprise expansion**

Phase	Typical duration	What happens
Discovery and workflow mapping	2 to 3 weeks	Define unit scope, staffing model, assignment rules, and rollout plan.
Configuration and training	2 to 4 weeks	Set up users, tabs, workflow expectations, and go-live support model.
Operational go-live without deep integration	4 to 8 weeks total	Fastest path when the unit can start with configured workflows and manual data entry.
Go-live with formal interface/integration work	10 to 14 weeks after kickoff, often inside a 4 to 6 month enterprise window	Consistent with Redox integration guidance and health-system expectations for new connected technologies.

Implementation timing above combines direct integration guidance from Redox with practical product rollout inference for this platform.

REVENUE MODEL**MRR, ARR, and year-one revenue scenarios**

Scenario	Hospitals live	MRR	ARR	Implementation revenue	Year-1 total revenue
Pilot	1	\$8,000	\$96,000	\$40,000	\$136,000
Regional cluster	3	\$20,000	\$240,000	\$100,000	\$340,000
System footprint	5	\$32,000	\$384,000	\$150,000	\$534,000
Area network scale	10	\$60,000	\$720,000	\$250,000	\$970,000

WHY THIS CAN SELL**Pricing sits well below plausible hospital value**

- A conservative modeled impact case from the earlier review was roughly \$438K in annual hospital value from modest turnover and safety-event improvement.
- That means a \$96K ARR single-hospital pilot price can still sit well below a realistic operational value range.
- At multi-hospital scale, the sales story becomes stronger because the product supports repeatable workflows, standardization, and shared system-level visibility.

Low-friction entry

Start as an operational workflow platform before full technical integration.

Expansion motion

Land in one unit or hospital, then expand to several sites in the same area or system.

INVESTOR TAKEAWAY

A practical path from pilot to regional footprint

- **Initial wedge:** one-hospital pilot with implementation plus subscription.
- **Expansion path:** regional rollout to 3 to 5 hospitals with standardized workflows.
- **System value:** recurring ARR from a multi-hospital footprint plus optional enterprise services.
- **Go-live expectation:** 4 to 8 weeks without deep integration, or roughly 10 to 14 weeks after kickoff when formal interface work is included.

Selected references: AHA hospital market context, AHA workforce financial pressure reporting, and Redox guidance on health-system integration timing. Pricing figures are inferred commercial strategy assumptions.